INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Degarelix: hormone injections every 28 days

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00481
Version 3

> Two starting injections; then every 28 days

Treatment stages

01 **Long-acting hormone treatment**
Two starting injections; then every 28 days

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your team promptly for new or worsening symptoms, severe low mood or problems taking treatment. New leg weakness, loss of bladder or bowel control, severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency assessment: call 112/999.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

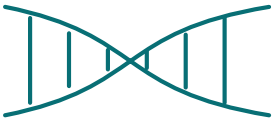
How this hormone treatment works

This is androgen deprivation therapy (ADT): it lowers testosterone to help control prostate cancer. With radiotherapy, the aim may be cure; in advanced disease, it is part of ongoing cancer control.

GNRH RECEPTOR ANTAGONIST

Degarelix

Firmagon



Directly blocks testosterone-stimulating signals

Degarelix is a GnRH receptor antagonist. It blocks the pituitary signal to the testes and lowers testosterone rapidly, without an initial testosterone flare. A depot under the skin releases the medicine between injections.

GnRH receptor blocked → Pituitary signal falls → Testosterone falls

What to watch for

Book the next injection before leaving. If an appointment is missed, contact the team promptly for a new date; do not arrange an extra loading dose yourself. Degarelix does not produce an initial testosterone surge, so anti-androgen treatment solely for flare prevention is not normally needed.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

What the evidence can tell you

Trials support the discussion but do not predict an individual outcome.

In the pivotal comparative study, sustained testosterone suppression from Day 28 to Day 364 occurred in 97.2% with the 240/80 mg degarelix regimen and 96.4% with leuprorelin. This supports hormone suppression; it does not prove a survival advantage or a cardiovascular advantage for an individual.

CHAPTER 02

Your schedule, tests and appointments

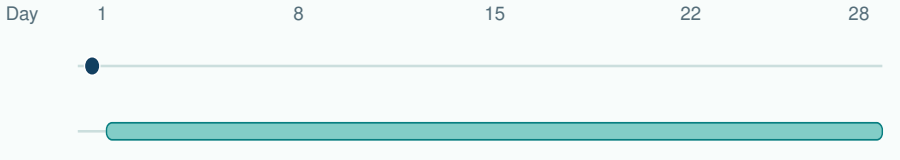
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment course

Degarelix: hormone injections every 28 days

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

TWO STARTING INJECTIONS; THEN EVERY 28 DAYS

Long-acting hormone treatment

Duration set by your treatment plan

The first dose is 240 mg as two 120 mg injections under the skin. The first 80 mg maintenance injection is 28 days later, then 80 mg every 28 days. The team sets the total duration.

Androgen deprivation therapy (ADT) lowers testosterone, a hormone that can drive prostate cancer. This medicine is your ADT. Additional cancer tablets, chemotherapy or radiotherapy are separate decisions based on your cancer and overall health. Keep treatment going until your team changes the plan; do not stop because prostate-specific antigen (PSA) has fallen.

BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Safety and response tests have different purposes. Use the individual prescription and appointment dates.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00481 v3: official regimen · Firmagon: product information

Tests in your plan

Blood count, kidney and liver tests

WHEN

Repeat blood count, kidney and liver tests, blood glucose and bone blood profile when clinically indicated. Review symptoms and the injection sites. PSA, testosterone and scans follow the individual cancer plan.

Checks blood cells and organ health.

Blood glucose and bone blood profile

WHEN

Repeat blood count, kidney and liver tests, blood glucose and bone blood profile when clinically indicated. Review symptoms and the injection sites. PSA, testosterone and scans follow the individual cancer plan.

Checks metabolic effects and blood minerals.

PSA and testosterone

If part of your plan

WHEN

Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.

Checks cancer activity and hormone suppression.

ECG and bone-density assessment

If part of your plan

WHEN

ECG (heart tracing) and bone-density scans are arranged according to heart-rhythm risk, fracture risk and clinical need. Your team confirms which tests you need.

Checks heart rhythm or fracture risk when indicated.

THE WHOLE TREATMENT JOURNEY

Degarelix: hormone injections every 28 days: treatment and checks

Adult men with advanced hormone-dependent prostate cancer, as covered by NCCP 00481.

The first dose is 240 mg as two 120 mg injections under the skin. The first 80 mg maintenance injection is 28 days later, then 80 mg every 28 days. The team sets the total duration.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	Between appointments	Next review	Longer-term care
Blood and organ safety NCCP · SMPC	Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal.	Confirm baseline results and the starting plan.	Repeat blood count, kidney and liver tests, blood glucose and bone blood profile when clinically indicated. Review symptoms and the injection sites. PSA, testosterone and scans follow the individual cancer plan.	Confirm results and symptoms before the next prescription or injection.	Review bone, metabolic and heart health.

Care / check	Before starting	First treatment	Between appointments	Next review	Longer-term care
Treatment and appointments NCCP · SMPC	Agree treatment aim, formulation, duration and medicines list.	A trained healthcare professional gives the injections under the skin of the abdomen. Injection sites are changed and kept away from pressure from belts or waistbands. This formulation is not given into a vein or muscle.	Keep an accurate medicine / injection record.	Book the next injection before leaving. If an appointment is missed, contact the team promptly for a new date; do not arrange an extra loading dose yourself. Degarelix does not produce an initial testosterone surge, so anti-androgen treatment solely for flare prevention is not normally needed.	Continue until the team changes the plan.
Cancer response NCCP · SMPC	Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.	Document the baseline PSA and clinical picture.	Ask when the next PSA and any testosterone test are due.	Review symptoms and results together.	Scans follow the individual plan; there is no universal interval.

What each check is for

1



Treatment and review

Two starting injections; then every 28 days

This is androgen deprivation therapy (ADT): it lowers testosterone to help control prostate cancer. With radiotherapy, the aim may be cure; in advanced disease, it is part of ongoing cancer control.

Ask / **act:** Repeat blood count, kidney and liver tests, blood glucose and bone blood profile when clinically indicated. Review symptoms and the injection sites. PSA, testosterone and scans follow the individual cancer plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before starting

Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal.

STEP 2

Taking or receiving treatment

A trained healthcare professional gives the injections under the skin of the abdomen. Injection sites are changed and kept away from pressure from belts or waistbands. This formulation is not given into a vein or muscle.

STEP 3

Tests and reviews

Repeat blood count, kidney and liver tests, blood glucose and bone blood profile when clinically indicated. Review symptoms and the injection sites. PSA, testosterone and scans follow the individual cancer plan.

STEP 5

Leaving with a clear plan

Confirm the exact next dose or appointment, duration and verified daytime and out-of-hours contacts.

Your exact treatment schedule

Tests and preparation

PSA and testosterone

Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Injection / implant visit	 **Day 1** The first dose is 240 mg as two 120 mg injections under the skin. The first 80 mg maintenance injection is 28 days later, then 80 mg every 28 days. The team sets the total duration.
Medicine continues working	 **Days 2–28** The depot keeps releasing medicine. Continue symptom checks; this is not a rest period.

Prescribed schedule: The first dose is 240 mg as two 120 mg injections under the skin. The first 80 mg maintenance injection is 28 days later, then 80 mg every 28 days. The team sets the total duration.

How this fits: Androgen deprivation therapy (ADT) lowers testosterone, a hormone that can drive prostate cancer. This medicine is your ADT. Additional cancer tablets, chemotherapy or radiotherapy are separate decisions based on your cancer and overall health. Keep treatment going until your team changes the plan; do not stop because prostate-specific antigen (PSA) has fallen.

Early safety: Book the next injection before leaving. If an appointment is missed, contact the team promptly for a new date; do not arrange an extra loading dose yourself. Degarelix does not produce an initial testosterone surge, so anti-androgen treatment solely for flare prevention is not normally needed.

> **Why these bands stay broad:** The interval between injections is not a treatment break. The depot keeps releasing medicine. The team confirms the exact next date. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00481 v3: official regimen ↗

> **There are no guaranteed "safe days".** Call your team promptly for new or worsening symptoms, severe low mood or problems taking treatment. New leg weakness, loss of bladder or bowel control, severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency assessment: call 112/999.

Between-treatment notes

Day 1: Treatment visit

A trained healthcare professional gives the injections under the skin of the abdomen. Injection sites are changed and kept away from pressure from belts or waistbands. This formulation is not given into a vein or muscle. The first dose is 240 mg as two 120 mg injections under the skin. The first 80 mg maintenance injection is 28 days later, then 80 mg every 28 days. The team sets the total duration.

Day 2–28: Between appointments

The depot continues working between injections. Symptoms can occur on any day. Keep your next booked appointment; contact the team promptly if it is missed.

Support around the medicines

1 **Sickness support**

This treatment has minimal sickness risk. Routine preventive anti-sickness medicines are not usually required; agree a plan if symptoms develop.

2 **Bone and muscle health**

Discuss fracture risk, bone-density testing when indicated, calcium intake, vitamin D and suitable resistance and aerobic exercise. Supplements and bone medicines are individual decisions.

3 **Sexual health, mood and hot flushes**

Tell the team which effects matter most. Support is available for sexual function, sleep, mood and vasomotor symptoms.

4 **Medicine review**

Bring all prescriptions, over-the-counter medicines and supplements. Ask pharmacy before starting or stopping anything.

Why hormone treatment is used

Treatment goals and choices

Additional treatment

In metastatic hormone-sensitive cancer, many suitable patients benefit from an additional androgen-pathway medicine, and sometimes chemotherapy. Radiotherapy can have a different role in localised disease, selected metastatic disease or symptom relief. Your specialist explains the options for your setting.

Deciding together

Discuss expected benefit, uncertainty, side effects, alternatives including surgical castration where appropriate, and the option of declining treatment. No one duration fits every setting.

Questions worth asking

Do I need another cancer medicine or radiotherapy alongside this?

Ask how this hormone treatment fits the plan for your disease. Additional hormone-pathway medicines, chemotherapy and radiotherapy have different roles; they are not automatically needed in every setting.

How can we protect bone, heart, sexual and mental health?

Ask for a plan covering exercise, bone and fracture risk, blood pressure and glucose, sexual symptoms, mood and sleep. Tell the team about changes early so support can be offered.

Who can I contact urgently?

Use the verified daytime and out-of-hours numbers supplied by your treating hospital. For severe breathing difficulty, collapse, a seizure or severe chest pain, call 112 or 999. Do not use the example numbers in this draft.

Try explaining the plan back: Explain your starting dose and usual schedule. Explain what to do after a missed dose or appointment. Name your next tests and your urgent contact route.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Two starting injections; then every 28 days

Active medicines: Degarelix

Emetogenic risk: Minimal

No routine prophylactic antiemetic order. Agree symptom treatment if needed.

Additional phase-specific support

- A trained healthcare professional gives the injections under the skin of the abdomen. Injection sites are changed and kept away from pressure from belts or waistbands. This formulation is not given into a vein or muscle.
- Repeat blood count, kidney and liver tests, blood glucose and bone blood profile when clinically indicated. Review symptoms and the injection sites. PSA, testosterone and scans follow the individual cancer plan.
- Book the next injection before leaving. If an appointment is missed, contact the team promptly for a new date; do not arrange an extra loading dose yourself. Degarelix does not produce an initial testosterone surge, so anti-androgen treatment solely for flare prevention is not normally needed.
- Bone health, calcium and vitamin D according to individual needs and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

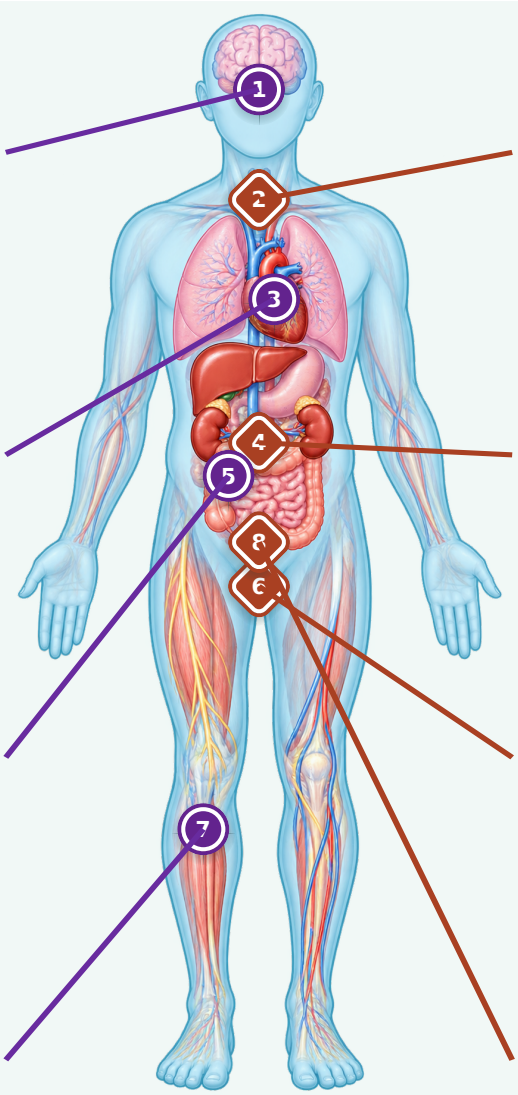
You do not need to know which medicine caused a symptom before calling.

1 Mood, sleep and energy
Degarelix
Report severe low mood, marked fatigue or sleep problems. Ask for support early.

2 Hot flushes and sweating
Degarelix
Hormone suppression can cause hot flushes. Ask about practical measures and treatments if troublesome.

3 Heart and blood pressure
Degarelix
Keep planned checks. Chest pain, fainting or severe breathlessness needs urgent assessment.

4 Weight and blood sugar
Degarelix
Blood glucose, weight and cardiovascular risk need review; suitable activity and diet can help.



5 Injection or implant site
Degarelix
Report worsening redness, swelling, severe pain or a persistent reaction.

6 Sexual health and fertility
Degarelix
Reduced desire and erection difficulties are common. Discuss fertility and contraception; ADT is not reliable contraception.

7 Bones and muscles
Degarelix
Ask about bone assessment, resistance exercise, falls prevention, calcium intake and vitamin D needs.

8 Urine and early symptoms
Degarelix
Report difficulty passing urine or worsening pain. New leg weakness or loss of bladder/ bowel control is an emergency.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call your team promptly for new or worsening symptoms, severe low mood or problems taking treatment. New leg weakness, loss of bladder or bowel control, severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency assessment: call 112/999.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00481 v3: official regimen**
   https://healthservice.hse.ie/documents/6496/481_v3_Degarelix_Therapy.pdf
2. **Firmagon: product information**
   https://www.ema.europa.eu/en/documents/product-information/firmagon-epar-product-information_en.pdf
3. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
4. **NCCP supportive-care antiemetics for NCIS**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **NCI: hormone therapy for prostate cancer**
   https://www.cancer.gov/types/prostate/prostate-hormone-therapy-fact-sheet